

DERMOCIN

MUPIROCIN OINTMENT BP 2.0% W/W

COMPOSITION:

Mupirocin Ph. Eur. 20 mg
In a water soluble base Q.S.

EXCIPIENTS: Macrogols-400 (Polyethylene glycol 400) Ph.Eur, Macrogols-4000 (Polyethylene glycol 4000) Ph.Eur.

PRODUCT DESCRIPTION:

White to off-white colour ointment.

PHARMACODYNAMICS:

Pharmacotherapeutic group: Antibiotics and chemotherapeutics for dermatological use.
ATC Code: D06AX09

Mode of Action:

Mupirocin is a novel antibiotic produced through fermentation by *Pseudomonas fluorescens*. Mupirocin inhibits isoleucyl transfer-RNA synthetase, thereby arresting bacterial protein synthesis. Mupirocin has bacteriostatic properties at minimum inhibitory concentrations and bactericidal properties at the higher concentrations reached when applied locally.

Mechanism of Resistance

Low-level resistance in staphylococci is thought to result from point mutations within the usual staphylococcal chromosomal gene (ileS) for the target isoleucyl tRNA synthetase enzyme. High-level resistance in staphylococci has been shown to be due to a distinct, plasmid encoded isoleucyl tRNA synthetase enzyme.

Intrinsic resistance in Gram negative organisms such as the *Enterobacteriaceae* could be due to poor penetration of the outer membrane of the Gram-negative bacterial cell wall.

Due to its particular mode of action, and its unique chemical structure, Mupirocin does not show any cross-resistance with other clinically available antibiotics.

Microbiological Susceptibility

The prevalence of acquired resistance may vary geographically and with time for selected species, and local information on resistance is desirable, particularly when treating severe infections. As necessary, expert advice should be sought when the local prevalence of resistance is such that the utility of the agent in at least some types of infection is questionable.

Commonly susceptible species: *Staphylococcus aureus**, *Streptococcus pyogenes**, *Streptococcus* spp. (β-haemolytic, other than *S. pyogenes*)

Species for which acquired resistance may be a problem: *Staphylococcus* spp., coagulase negative

Inherently resistant organisms: *Corynebacterium* spp., *Micrococcus* spp.

* Activity has been satisfactorily demonstrated in clinical studies.

PHARMACOKINETIC:

After topical application of Mupirocin Ointment, Mupirocin is only very minimally absorbed systemically and that which is absorbed is rapidly metabolised to the antimicrobially inactive metabolite, monic acid. Penetration of Mupirocin into the deeper epidermal and dermal layers of the skin is enhanced in traumatised skin and under occlusive dressings

Elderly patients

No restrictions unless there is evidence of moderate or severe renal impairment

INDICATIONS:

Mupirocin Ointment is used for bacterial skin infections, e.g. impetigo, folliculitis and furunculosis.

RECOMMENDED DOSE:

Adults and children:

Mupirocin Ointment should be applied to the affected area 2 to 3 times a day, for up to 10 days, depending on the response.

The area may be covered with a dressing or occluded if desired.

Do not mix with other preparations as there is a risk of dilution, resulting in a reduction of the antibacterial activity and potential loss of stability of the Mupirocin in the ointment.

ROUTE OF ADMINISTRATION: Topical

CONTRAINDICATIONS:

Mupirocin Ointment should not be given to patients with a history of hypersensitivity to any of its constituents.

This Mupirocin Ointment formulation is not suitable for ophthalmic or intranasal use.

WARNINGS AND PRECAUTIONS:

'Jauhkan daripada capaian kanak-kanak / Keep out of reach of children'

'Please consult your pharmacist/doctor before taking this product'

Should a possible sensitisation reaction or severe local irritation occur with the use of Mupirocin ointment, treatment should be discontinued, the product should be washed off and appropriate therapy instituted.

As with other antibacterial products, prolonged use may result in overgrowth of non-susceptible organisms.

Pseudomembranous colitis has been reported with the use of antibiotics and may range in severity from mild to life-threatening. Therefore, it is important to consider its diagnosis in patients who develop diarrhoea during or after antibiotic use. Although this is less likely to occur with topically applied Mupirocin, if prolonged or significant diarrhoea occurs or the patient experiences abdominal cramps, treatment should be discontinued immediately and the patient investigated further.

Renal impairment:

Polyethylene glycol can be absorbed from open wounds and damaged skin and is excreted by the kidneys. In common with other polyethylene glycol based ointments, Mupirocin ointment should not be used in conditions where absorption of large quantities of polyethylene glycol is possible, especially if there is evidence of moderate or severe renal impairment.

Mupirocin ointment is not suitable for: Ophthalmic use, Intra-nasal use, use in conjunction with cannulae and at the site of central venous cannulation.

Avoid contact with the eyes. If contaminated, the eyes should be thoroughly irrigated with water until the ointment residues have been removed.

INTERACTION WITH OTHER MEDICAMENTS:

No drug interactions have been identified.

PREGNANCY AND LACTATION:

Pregnancy: Reproduction studies on Mupirocin ointment in animals have revealed no evidence of harm to the foetus. As there is no clinical experience on its use during pregnancy, Mupirocin ointment should only be used in pregnancy when the potential benefits outweigh the possible risks of treatment.

Lactation: There is no information on the excretion of Mupirocin in milk. If a cracked nipple is to be treated, it should be thoroughly washed prior to breast feeding.

Fertility: There are no data on the effects of Mupirocin on human fertility. Studies in rats showed no effects on fertility.

SIDE EFFECTS:

Immune system disorders:

Very rare: Systemic allergic reactions including anaphylaxis, generalised rash, urticaria and angioedema have been reported with Mupirocin ointment.

Skin and subcutaneous tissue disorders:

Common: Burning localised to the area of application.

Uncommon: Itching, erythema, stinging and dryness localised to the area of application.

Cutaneous sensitisation reactions to Mupirocin or the ointment base.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptoms and signs:

There is currently limited experience with over dosage of

Mupirocin.

Treatment: There is no specific treatment for an overdose of Mupirocin. In the event of overdose, the patient should be treated supportively with appropriate monitoring as necessary. Further management should be as clinically indicated or as recommended by the national poisons centre, where available.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE:

No adverse effects on the ability to drive or operate machinery have been identified.

PRECLINICAL SAFETY DATA:

Not Applicable

INSTRUCTION FOR USE:

Mupirocin Ointment should be applied to the affected area 2 to 3 times a day, for up to 10 days, depending on the response.

The area may be covered with a dressing or occluded if desired.

Do not mix with other preparations as there is a risk of dilution, resulting in a reduction of the antibacterial activity and potential loss of stability of the Mupirocin in the ointment.

Any ointment remaining at the end of treatment should be discarded.

Wash your hands after application.

STORAGE CONDITION:

Do not store above 30°C. Do not freeze.

The product should not be used more than 28 days after opening of the tube.

SHELF LIFE:

24 months

The product should not be used more than 28 days after opening of the tube.

PRESENTATION:

White to off-white colour ointment is filled in 5 gm or 15 gm white Aluminum Collapsible Tube and closed with crock wadded white cap (with piercing point). Such 1 tube packed in printed carton with package insert.

Manufactured by :



Trimul Estate, Khatraj, Tal.- Kalol,
Dist.- Gandhinagar, Gujarat State,
India

E-mail : info@lincolnpharma.com

Website : www.lincolnpharma.com

Product Registration Holder:

Heald Pharmaceuticals Sdn. Bhd.
74-3, Jalan Wangsa Delima 6,
KLSC Wangsa Maju,
53300 Kuala Lumpur, Malaysia.

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